NCCP 00414 · VERSION 5A

Palbociclib with endocrine therapy

The usual palbociclib starting dose is 125 mg once daily; a lower dose may be prescribed. Palbociclib is taken once daily on Days 1–21, then paused on Days 22–28 of each 28-day cycle. The aromatase inhibitor continues daily, or fulvestrant follows its injection schedule. Ovarian suppression has its own plan when required.

> Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Menopause and ovarian-suppression requirements if relevant

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Low neutrophils and infection risk

☐ Mouth soreness

☐ Hair thinning

☐ Hot flushes and other hormone-related symptoms

☐ Fulvestrant: injection-site pain and bruising

☐ Fatigue

☐ Nausea or diarrhoea

☐ Rare but serious interstitial lung disease / pneumonitis

☐ AI: joint symptoms and bone loss

☐ Partner-specific liver, interaction and pregnancy considerations

Discuss the treatment aim, alternatives, benefit and uncertainty, important medicine-specific risks, reproductive precautions, symptom support and planned duration. Confirm the endocrine partner and any ovarian-suppression plan. The timeline does not authorise a restart or certify clinical review.

Individual benefit, alternatives and concerns

Agreed checks and next review

FBC, renal and liver profile. Confirm receptor status, exact endocrine partner, previous endocrine therapy, menopause/ovarian suppression, pregnancy precautions and interacting medicines. FBC on Day 1 and Day 15 of cycles 1–2; Day 1 from cycle 3 and extra checks as indicated. Only after at most grade 1–2 neutropenia throughout the first six cycles may FBC monitoring reduce to every three months, before the start of a cycle and as indicated. Renal and liver profile before each cycle; review fever/infection, mouth/bowel symptoms, fatigue, pulmonary symptoms and adherence. No routine palbociclib-specific Day-14 ECG or echo timetable; investigate when clinically indicated.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.21 · Sources checked 2026-10-25
Source reconciled; independent clinical, pharmacy and patient/PPI review pending